
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the one small thing we'd actually have you do first.

PREPARED FOR — TYPE C PRACTICE

Practices with an EHR and a workflow that already works.

Your system works. You have a back room of paper. That is the whole gap.

Written specifically for a practice in your situation.
The first step we recommend is the same for every practice — and it is deliberately the smallest one.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

Your practice is already digital and your workflow already hangs together — so we are not going to pretend you need a new platform, and we are not going to. The demo was deliberately narrow: your legacy archive becoming structured data, into the system you already run. That is the gap. That is all the demo was about.

— Your archive, structured, into the system you already run

We took your own legacy files, extracted them into structured records, and showed the export landing in the format your EHR ingests. No change to your system of record.

— The original scan, always one click away

Every structured record stays linked to the page it came from. Nothing is a black box; the source is always retrievable.

— Nothing about your working setup disturbed

No platform migration, no workflow change, no retraining. A layer on top of what already works — and only the layer you actually have a gap for.

02 WHAT THIS ACTUALLY COSTS YOU IN STAFF TIME

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

Per document

30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.

Week one

Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.

Month three

The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.

What does not change

How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

The same first step regardless of how your practice is set up today — because every practice, paper or EHR or fully wired, has the same one gap that digitisation closes: a back room of legacy files that is dead weight until it is structured data.

Digitisation only. Nothing else, to start.

Not a platform. Not analytics. Not a bundle. The single thing we would put in your practice first is the digitisation of your legacy archive — because it is the one thing every practice needs, it is complete and useful entirely on its own, and it changes nothing about how you already work.

A real, complete product — not a foot in the door

Digitisation is not a teaser for something else. A practice that only ever digitises, and never buys another module, has still solved a real problem permanently. We mean that; the rest of this document is written so you can hold us to it.

Priced as a subscription, not a project

A fixed monthly page allowance, payable and cancellable, no six-figure project quote. We work through the archive at that pace. It takes time — and the predictability is the point, for you and for us.

Everything else this company sells is downstream of this one step — and it is downstream for a concrete reason, not a commercial one. See the next section.

This is the single most important thing in this document, so we are going to state it as the fact it is, not as a sales line. Analytics cannot run on a filing cabinet. The platform's value does not compound without structured history. Every capability beyond digitisation — the view of your practice, the cohorts, the search, anything further — needs the structured data that digitisation, and only digitisation, produces. That is not a reason to buy more. It is the reason the order is fixed: the archive becomes a data asset first, and whatever you choose to do with that asset comes later, on your timing. A vendor that sold you the downstream modules before the data existed would be selling you something that cannot work yet.

We want to be careful here, because a practice in your situation is right to be wary of an upsell. So plainly: this section is not an offer. It is here only so you know what the structured data *makes possible* later — entirely at your initiative, never pushed.

Once the archive is structured — the analytics view

If you ever want the cross-practice view your EHR stores the data for but cannot surface — chronic cohorts, claims aging, semantic search across the whole archive — it becomes available the moment the data is structured. It needs nothing but the digitisation output. It is the one expansion that is genuinely natural for a practice like yours, and even this is yours to ask for, not ours to push.

The platform — deliberately not on this path

We are explicitly *not* putting the practice platform on your expansion path. Your workflow works. Selling you a platform you do not need would be the wrong large thing. If a Prognos person ever pitches you one, that is a mistake on our side, and we would want to know.

For a practice like yours the honest position is: digitisation solves your only real gap, completely, possibly forever. Analytics is a natural option later if you want it. The platform is not on the table and we are not going to put it there. We would rather have a long-term relationship with a practice that trusts us than a short one with a practice we over-sold.

The same discipline we apply to the product, applied to this document.
These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in this document, treat it as not yet real.

Not the catalogue. Not your type's eventual configuration. One small, bounded, cancellable first step — the same one we'd recommend to any practice, because it is the one that earns everything after it.

THE SMALLEST FIRST STEP

One month. Your files. Digitisation only.

Not the whole archive. Not a year's commitment. Not a bundle.

Digitisation only: one month, 1,500 pages of your choosing — ideally the messiest ones, so you are testing the hard case. Your staff validates as they would in production; the output lands in whatever system you already run, or in ours if you have none. At the end of the month you have a real, structured slice of your own archive and an honest sense of the day-to-day. No setup fee, no lock-in, cancel any time. Everything else is a later conversation that this step earns — or doesn't.

Prognosis Labs

luzuko@medicdata.co.za · +27 (0) 76 969 5462

medicdata.co.za · book a follow-up — medicdata.co.za/demo